

Venus K-Glow Boosters

Doctor Training Knowledge Evaluation - Candidate Question Paper | Version B

Assessment code	VA-KGLOW-B-2026.08
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Candidate and assessment details

Doctor full name		Employee ID	
Branch / city		Assessment date	
Trainer		Examiner	
Start time		Finish time	

Assessment structure and instructions

Section	Format	Questions	Scoring
A	Core and applied MCQs	40	80 marks - 2 each
B	Written clinical questions	5	20 marks - 4 each
C	Advanced Distinction MCQs	5	Separate score /5

Time allowed: 90 minutes. This is a closed-book assessment unless the examiner states otherwise. Select one best answer for every MCQ and mark it on the separate candidate answer sheet. Written answers should be concise, specific and clinically relevant. Mobile phones, smart watches, notes, slides, manuals, AI tools and discussion with other candidates are not permitted. The Advanced Distinction score is separate and does not determine the basic pass result.

Section A - Core and Applied MCQs (Questions 1-40 | 80 marks)

1. A patient requests K-Lumi Eyes because of dark circles. Examination shows mainly epidermal pigmentation, with no crepiness, dehydration, thin skin or fine lines. What is the best response?

- A. Proceed with K-Lumi because every type of dark circle responds to PN.
- B. Use a higher-volume K-Lumi treatment to compensate for the pigmentary cause.
- C. Explain that pigment is the dominant concern and direct the patient to a pigment-targeted plan; consider K-Lumi only if a separate under-eye skin-quality indication is present.
- D. Replace K-Lumi with K-Glass BAP injections under the eyes.

2. A patient has strong dynamic glabellar lines and also complains of oily skin and enlarged pores. Which plan best follows the Venus framework?

- A. Use K-Tox Refine alone and promise the same muscle-freezing result as conventional Botox.
- B. Assess conventional neuromodulator treatment for the dynamic lines and consider K-Tox Refine separately for pore, sebum and surface-quality concerns.
- C. Use K-Glass Hydration because all facial lines are caused by dehydration.
- D. Use K-Colla Bounce as an immediate substitute for both Botox and pore treatment.

3. A doctor remembers a product from an older training slide, but it is not listed on the current clinic price sheet. What should happen before it is prescribed?

- A. Prescribe it because any product previously mentioned remains permanently approved.
- B. Use it only for staff members until new stock arrives.
- C. Verify the current Venus-approved product list and protocol with the current price sheet or management before prescribing or using it.
- D. Write the old product name but substitute whichever product is available at treatment time.

4. A patient had an aggressive peel four days ago and still has diffuse erythema, tightness and flaking. She requests a microneedling-assisted K-Glow booster today. What is the best decision?

- A. Proceed at maximum depth because the peel has already opened the skin.
- B. Proceed only on visibly dry areas.
- C. Delay treatment until the skin has recovered, because a recent aggressive procedure and active irritation increase risk.
- D. Change to an injectable booster so the epidermis is bypassed.

5. During a K-Lip Dew consultation, the patient reports new tingling at the lip border and has a history of recurrent cold sores. What should the doctor do?

- A. Proceed because the booster contains hydrating HA.
- B. Proceed after applying more numbing cream.
- C. Delay the treatment, assess for an evolving herpes outbreak and manage or obtain appropriate medical advice before reconsidering.
- D. Inject only the central lip and avoid the tingling area.

6. A patient with poorly controlled diabetes is taking immunosuppressive medication and requests K-Exo Glow after seeing an online advertisement. What is the most appropriate approach?

- A. Proceed because exosomes are cell-free and therefore cannot cause complications.
- B. Proceed only with a lower Dermapen speed.
- C. Delay treatment and obtain appropriate medical and senior review because uncontrolled systemic disease and immunosuppression increase procedural risk.
- D. Use K-PDRN instead because it is derived from DNA.

7. A supplier representative says an exosome preparation may be injected, but the product documentation available in the clinic only supports post-procedure topical use. What should the doctor do?

- A. Inject it because the supplier verbally confirmed the route.
- B. Inject a small test dose first.
- C. Do not inject it; use only a route supported by verified product documentation and the current Venus protocol.
- D. Mix it with saline so the route becomes acceptable.

8. A sterile Dermapen cartridge is opened, attached and then accidentally touches a non-sterile trolley before the procedure begins. What is the correct action?

- A. Wipe the cartridge with alcohol and continue.
- B. Use it only on the forehead because that area is less sensitive.
- C. Discard it and attach a new sterile cartridge.
- D. Keep it for the same patient at the next session.

9. An assistant hands the doctor an unlabeled prefilled syringe, stating that it is the product sold to the patient. What should the doctor do before treatment?

- A. Proceed if the syringe colour looks familiar.
- B. Ask the patient whether the product name sounds correct.
- C. Stop and verify the exact prescribed product, original packaging, integrity, batch, expiry and quantity before using it.
- D. Use half the syringe first and verify later if results are poor.

10. Three days after microneedling-assisted K-Colla Bounce, a patient reports increasing warmth, spreading redness, pus and fever. What is the correct interpretation?

- A. Normal collagen-induction downtime that requires reassurance only.
- B. Expected flaking that should be treated with a facial scrub.
- C. A possible infection requiring urgent clinical assessment and escalation.
- D. A normal reaction that proves the product reached the dermis.

11. Why does a non-cross-linked HA skin booster generally behave differently from a cross-linked HA filler?

- A. Non-cross-linked HA is designed mainly for water binding and tissue-quality improvement, while cross-linking creates a more degradation-resistant gel suited to filling and structural support.
- B. Non-cross-linked HA permanently freezes muscle movement.
- C. Cross-linked HA is always topical, while non-cross-linked HA is always injectable.
- D. There is no clinically relevant difference once HA enters the skin.

12. Which K-Glass product description is fully correct according to the training table?

- A. Profhilo: 64 mg non-cross-linked HA in 2 mL; Lumi Pro: 40 mg non-cross-linked HA in 2 mL; Lamist: 68 mg non-cross-linked HA plus succinic acid in 2 mL.
- B. Profhilo: 20 mg PN in 2 mL; Lumi Pro: 64 mg cross-linked HA; Lamist: recombinant collagen only.
- C. Profhilo: 40 mg cross-linked HA; Lumi Pro: 68 mg HA plus CMC; Lamist: 2% PDRN.
- D. All three products have identical composition and differ only by country of origin.

13. A patient has marked dehydration plus early loss of elasticity and wants the premium K-Glass option that has a broader tissue-quality objective. Which product best matches the taught selection logic?

- A. Profhilo, because it is positioned for hydration plus bioremodelling and tissue-quality improvement.
- B. Lumi Meso Tox, because pores and sebum determine elasticity.
- C. Lapuroon Vivid, because all DNA-derived products act as HA.
- D. Baccio Lip Booster, because vitamin B12 stimulates facial volume.

14. How is the nasal-base BAP point located in the marking guide?

- A. At the midpoint of the nasolabial fold, independent of other landmarks.
- B. At the intersection of a line from the nostril toward the tragus and a vertical line from the pupil.
- C. Two centimetres directly below the medial canthus.
- D. At the lateral corner of the mouth.

15. Which method best reflects the taught chin BAP marking approach?

- A. Mark the centre of the lower lip and inject directly on the midline.
- B. Create a chin midline, identify the upper third, then mark approximately 1.5 cm laterally from the relevant intersection on each side.
- C. Mark directly over the mental foramen without palpation.
- D. Use the mandibular angle point for the chin as well.

16. Which sequence is most consistent with the Venus K-Glass BAP workflow?

- A. Open the syringe, inject by visual estimation, then mark any missed points.
- B. Mark the ten points first, verify the product, attach and prime the needle, orient the bevel appropriately, inject the complete 2 mL slowly in controlled 0.2 mL deposits and observe the skin.
- C. Dilute the syringe with saline, use twenty points and store any remainder.
- D. Inject one side only and use the second half at the next visit.

17. Before starting a K-Glass HA injection, the doctor discovers that the treatment room has no accessible hyaluronidase and the emergency escalation contact cannot be reached. What should happen?

- A. Proceed because vascular compromise is uncommon with non-cross-linked HA.
- B. Proceed with half the dose and arrange the emergency supplies later.
- C. Postpone the injectable treatment until the required emergency medication, supportive equipment and escalation pathway are available.
- D. Replace hyaluronidase with a cold compress.

18. Immediately after a BAP deposit, a small soft papule forms without severe pain, colour change or visual symptoms. What is the best interpretation?

- A. This alone is an expected temporary reaction that should settle; continue monitoring and counsel the patient.
- B. It proves arterial occlusion and requires hospital transfer.
- C. It means the complete syringe must be dissolved.
- D. It shows the product was injected as a filler and must be massaged aggressively.

19. After a Dermapen-assisted booster, which advice is most consistent with the universal Venus post-care?

- A. Resume retinol and exfoliating acids the same evening to enhance penetration.
- B. Avoid touching and makeup initially, avoid heat and strenuous exercise for about 24-48 hours, use gentle skincare and sunscreen, avoid strong actives for several days and other irritating procedures for roughly 1-2 weeks unless advised.
- C. Scrub the skin daily to remove flaking.
- D. Book an aggressive peel within 48 hours while microchannels remain open.

20. In the K-Colla Bounce name, what does "bounce" describe?

- A. Immediate structural volume and cheek projection.
- B. Improved elasticity, firmness and the skin's ability to recover its shape or resilience.
- C. Temporary oedema after treatment.
- D. Complete removal of acne scars.

21. Which product-route pairing is correct under the current K-Colla training?

- A. SeduSkin Hylagen3 - inject by BAP; Rejuroll - intramuscular; Karisma - Dermapen only.
- B. SeduSkin Hylagen3 - Venus microneedling protocol and do not inject; Rejuroll - current Venus microneedling protocol despite manufacturer injection options; Karisma - product-specific injectable technique, not routine microneedling.
- C. All collagen products use the same technique because the ingredient category determines the route.
- D. All three products are topical masks.

22. A doctor is using a 2.5 mL Rejuroll syringe through the current Venus microneedling protocol. Which product-management approach best matches the verbal training?

- A. Use the full syringe on the first cheek and substitute saline on the second.
- B. Plan approximately equal distribution, around 1.25 mL per side, while observing coverage and the approved endpoint.
- C. Use 0.5 mL and save the rest for other patients.
- D. Mix it with PRP to increase the volume to 5 mL.

23. A patient with a darker skin type and a strong history of post-inflammatory hyperpigmentation is suitable for K-Colla microneedling. How should the procedure be modified?

- A. Use more passes and deeper settings to overcome melanin activity.
- B. Use conservative depth and passes, maintain controlled erythema rather than aggressive bleeding, and reinforce post-care and sun protection.
- C. Create uniform bleeding because this prevents PIH.
- D. Combine an aggressive peel in the same visit.

24. Which comparison between PN and PDRN is most consistent with the training?

- A. PN generally has longer DNA chains and a more viscoelastic support emphasis; PDRN contains shorter fragments and has a lighter repair/recovery emphasis.
- B. PN is cross-linked HA, while PDRN is collagen.
- C. PN and PDRN are identical terms with no difference in chain length or clinical emphasis.
- D. PDRN is a live stem-cell preparation and PN is fish oil.

25. Which product profile is correctly matched in the K-PDRN Reset table?

- A. Rejuran Healer: 2 mL syringe with 2% PN; Lapuroon Aurora Super: 2.5 mL syringe with 2% PDRN; Lapuroon Aurora Vivid: 5 mL vial with 0.2% PDRN.
- B. Rejuran Healer: 5 mL exosome vial; Lapuroon Super: cross-linked HA filler; Vivid: botulinum toxin.
- C. All three contain the same volume and concentration.
- D. Rejuran is a lip booster, Super is an eye filler and Vivid is a collagen implant.

26. During K-PDRN Reset, an assistant proposes holding the syringe and dropping product while the doctor uses the Dermapen. What does the verbal training prefer?

- A. The doctor should control both product droplets and microneedling delivery so distribution and remaining volume can be monitored directly.
- B. The assistant should inject the product while the doctor microneedles.
- C. The product should be poured over the full face before cleansing.
- D. The product should be mixed into numbing cream.

27. Which K-PDRN plan follows the taught Venus protocol?

- A. Mix PDRN with PRP and saline in one bowl to improve spread.
- B. Use controlled Dermapen-assisted delivery, or topical application after RF microneedling where protocol allows, without casual mixing with PRP, saline or other boosters.
- C. Inject all facial PDRN products by the BAP technique.
- D. Use aggressive bleeding as the endpoint.

28. A patient with deep tethered acne scars asks whether K-PDRN Reset alone will cure the scars. What is the best counselling?

- A. Promise complete scar removal after one session.
- B. Explain that K-PDRN may support skin quality and recovery but is not a cure for deep scars; scar-specific treatment such as RF microneedling, subcision or laser may be the primary plan.
- C. Use a higher PDRN concentration and avoid scar-specific treatment.
- D. Replace PDRN with K-Lip Dew on the scars.

29. Which statement best describes the role of exosomes in the K-Exo module?

- A. They are living stem cells that permanently replace damaged skin cells.
- B. They are extracellular signalling vesicles carrying bioactive cargo; they support cell-to-cell communication but do not become new cells.
- C. They are cross-linked HA particles used for volume.
- D. They are a form of injectable Botox.

30. Which comparison of the two K-Exo product options is correct according to the training?

- A. Exoxe: made in Korea, 6 billion exosomes, described as amniotic-fluid stem-cell derived; ASCE+ ExCoBio: made in Korea, 5 billion exosomes, described as adipose-tissue derived.
- B. Exoxe: 5 billion adipose-derived; ASCE+: 6 billion salmon-derived.
- C. Both are PN injections from salmon sperm cells.
- D. Both are cross-linked HA fillers supplied in 2 mL syringes.

31. What is the taught preparation and skin-delivery approach for the lyophilized K-Exo products?

- A. Inject the dry powder directly before adding diluent.
- B. Reconstitute the lyophilized vial with the supplied diluent for the skin preparation, then use the verified Dermapen-assisted or post-RF topical protocol; inject only if explicitly supported.
- C. Mix the powder with a strong acid peel.
- D. Use normal saline for every skin and hair protocol without checking product instructions.

32. Which statement is most appropriate when counselling and documenting K-Exo Glow?

- A. Guarantee DNA repair and reversal of cellular ageing because the vial has billions of particles.
- B. Call it stem-cell therapy to simplify the explanation.
- C. Explain that it is a signalling and recovery-support treatment with variable evidence and product quality; verify source, preparation, storage, batch and technique and avoid disease-cure claims.
- D. State that particle count alone proves clinical superiority.

33. Which patient is the strongest candidate for K-Lumi Eyes?

- A. A patient with severe fat prolapse and large eye bags requesting their removal.
- B. A patient with thin, mildly dehydrated, crepey under-eye skin and fine superficial lines, without major bags or unresolved swelling.
- C. A patient with active periocular dermatitis.
- D. A patient with major tear-trough structural hollowing who refuses filler assessment.

34. Which shortlisted under-eye product pairing is correct in the training?

- A. Lumi Pro Eyes: 5 mg in a 1 mL syringe; AMI Eyes: 20 mg in a 2 mL syringe.
- B. Lumi Pro Eyes: 64 mg HA in 2 mL; AMI Eyes: 5 billion exosomes.
- C. Lumi Pro Eyes: 5 mL mesotox vial; AMI Eyes: 2.5 mL lip booster.
- D. Both are cross-linked volumizing fillers.

35. Which technique description best matches the taught K-Lumi injection principles?

- A. Deep orbital boluses using the full 4 mm needle perpendicular to the skin.
- B. Superficial papillary-dermal microinjections using conservative volume, approximately a 10-20 degree angle and the approved 32G x 4 mm mesotherapy needle, while avoiding deep orbital placement and the lash line.
- C. Intramuscular injection into the orbicularis to freeze movement.
- D. BAP injection at the five facial points.

36. After an under-eye PN injection, a patient develops severe swelling, eye pain and blurred vision. What is the best response?

- A. Reassure the patient that all under-eye puffiness is normal.
- B. Inject hyaluronidase and discharge immediately because it dissolves PN.
- C. Treat this as an urgent eye-related complication, stop treatment, assess and escalate for immediate medical or ophthalmic review; remember that hyaluronidase does not dissolve pure PN/PDRN.
- D. Massage the orbit firmly until vision returns.

37. Which finding is the clearest reason to delay K-Lip Dew?

- A. Dry lips with fine surface lines and no active disease.
- B. A settled lip filler placed three months ago with healthy tissue.
- C. Active cheilitis with fissuring and a possible dental infection.
- D. A patient who wants hydration rather than volume.

38. Which K-Lip Dew statement is correct according to the training?

- A. Skinfill Baccio is a 2.5 mL non-cross-linked HA 20 mg/mL plus vitamin B12 lip booster used with the approved stamp needle; Lumi Pro Lips is a 1 mL 22 mg/mL non-cross-linked HA option that can use the same approved pink stamp-needle approach.
- B. Baccio is a cross-linked Russian-lip filler and Lumi Pro Lips is injectable Botox.
- C. Both products must be shared between patients because the lip requires only 0.2 mL.
- D. The pink colour guarantees permanent pink pigmentation of the lips.

39. A patient requests K-Lip Dew seven days after lip filler. Examination shows persistent swelling, migration and tender nodules. What is the best plan?

- A. Proceed because a booster will smooth and dissolve the migration.
- B. Delay the booster, assess and correct the filler-related problem first, and reconsider only after the lips are clinically healthy and settled.
- C. Use more booster on the migrated side to restore symmetry.
- D. Proceed if the patient signs that results are not guaranteed.

40. A patient with oily skin and enlarged pores asks for K-Tox Refine and expects the forehead to be completely frozen. Which counselling and protocol statement is most accurate?

- A. Lumi Meso Tox is injected intramuscularly exactly like standard Botox and guarantees muscle paralysis.
- B. K-Tox Refine uses the complete 5 mL Lumi Meso Tox vial through the approved superficial microneedling protocol for pore, sebum and surface refinement; it is not a substitute for injectable Botulinum Toxin Type A, and possible problems include irritation, dryness, uneven refinement or eyebrow heaviness.
- C. K-Tox is a collagen filler used for volume loss.
- D. The vial should be divided among five patients to prevent heaviness.

Section B - Written Clinical Questions (Questions 41-45 | 20 marks)

Answer each question on the separate written-answer sheet. Each question is worth 4 marks. Four clear clinical points are usually sufficient.

41. Protocol reconstruction - K-PDRN Reset with Dermapen

A suitable patient has selected facial K-PDRN Reset using the current Venus microneedling-assisted protocol. Write four essential steps or controls from consultation through completion.

42. Patient selection - lip booster after filler with herpes symptoms

A patient requests K-Lip Dew ten days after lip filler. She has persistent swelling, a migrated lump and new tingling at the lip border. She also expects the booster to add volume and make the lips permanently pink. State four elements of your decision and counselling.

43. Product preparation and claims - K-Exo Glow

The clinic has received a lyophilized exosome skin product with a supplied diluent. Write four points covering verification, preparation, delivery and patient counselling before K-Exo Glow.

44. Urgent management - visual symptoms after K-Lumi Eyes

During an under-eye PN treatment, the patient develops severe swelling, eye pain and blurred vision. State the first four actions or decisions required.

45. Integrated planning - acne-scar and recovery programme

A patient has completed control of active acne and is starting a Venus Viva/RF-microneedling scar programme. She asks for K-PDRN Reset, K-Colla Bounce and K-Exo Glow in the same visit because she wants faster results. Propose a safe four-point plan.

Section C - Advanced Distinction MCQs (Questions 46-50 | Separate score /5)

These questions assess relevant knowledge beyond the exact presentation. They do not reduce the core score, but at least 4/5 is required for Pass with Distinction.

46. What is the most evidence-based interpretation of a negative aspiration before an HA injection?

- A. It proves the needle is outside a blood vessel and makes a large bolus safe.
- B. It cannot reliably exclude intravascular placement; anatomical knowledge, small controlled doses, slow low-pressure injection and continuous observation remain essential.
- C. It eliminates the need for hyaluronidase availability.
- D. It is reliable only when the patient is fasting.

47. In a randomized, double-blind, split-face periocular study comparing PN with non-cross-linked HA, which broad result was reported?

- A. Neither treatment changed hydration or elasticity.
- B. Both groups improved, but the PN side showed a higher improvement rate in elasticity and hydration over time.
- C. HA caused permanent muscle paralysis.
- D. PN produced immediate structural volume equivalent to a cross-linked filler.

48. Which conclusion is most appropriate from a 2025 randomized controlled trial of injectable recombinant humanized type III collagen for facial rejuvenation?

- A. The preliminary trial reported improved aesthetic scores, but results apply to the studied formulation and do not prove that every collagen-containing product or route is equivalent.
- B. Any topical product containing the word collagen can therefore be injected.
- C. Recombinant collagen permanently replaces all native collagen after one treatment.
- D. The trial proves that collagen boosters are substitutes for surgery in advanced laxity.

49. Which formulation matches the published 2024 clinical study of Karisma Rh Collagen FACE?

- A. Non-cross-linked high-molecular-weight HA, a recombinant collagen-1 alpha-chain polypeptide and carboxymethyl cellulose.
- B. Cross-linked HA, salmon PDRN and botulinum toxin type A.
- C. Calcium hydroxylapatite, PLLA and lidocaine only.
- D. Adipose-derived extracellular vesicles and tranexamic acid.

50. Why is an exosome label such as "5 billion particles" insufficient by itself to establish product quality or potency?

- A. Because extracellular-vesicle assessment also requires information about source material, separation and purity, size and molecular characterization, handling and storage, and functional evidence.
- B. Because particle count is legally prohibited in all countries.
- C. Because every particle measured is automatically a live stem cell.
- D. Because lower particle counts always produce better outcomes.

End of Question Paper. Confirm that Questions 1-50 have been attempted before submitting your answer sheet.

Venus K-Glow Boosters

Candidate Answer Sheet | Version B

Assessment code	VA-KGLOW-B-2026.08
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Candidate details

Doctor full name		Employee ID	
Branch / city		Assessment date	
Start time		Submission time	

Section A - MCQ answer grid

Place one clear tick or circle in the selected option. Multiple marked answers will be treated as incorrect unless the examiner authorizes a correction.

Q	A	B	C	D	Q	A	B	C	D	Q	A	B	C	D	Q	A	B	C	D
1	☐	☐	☐	☐	11	☐	☐	☐	☐	21	☐	☐	☐	☐	31	☐	☐	☐	☐
2	☐	☐	☐	☐	12	☐	☐	☐	☐	22	☐	☐	☐	☐	32	☐	☐	☐	☐
3	☐	☐	☐	☐	13	☐	☐	☐	☐	23	☐	☐	☐	☐	33	☐	☐	☐	☐
4	☐	☐	☐	☐	14	☐	☐	☐	☐	24	☐	☐	☐	☐	34	☐	☐	☐	☐
5	☐	☐	☐	☐	15	☐	☐	☐	☐	25	☐	☐	☐	☐	35	☐	☐	☐	☐
6	☐	☐	☐	☐	16	☐	☐	☐	☐	26	☐	☐	☐	☐	36	☐	☐	☐	☐
7	☐	☐	☐	☐	17	☐	☐	☐	☐	27	☐	☐	☐	☐	37	☐	☐	☐	☐
8	☐	☐	☐	☐	18	☐	☐	☐	☐	28	☐	☐	☐	☐	38	☐	☐	☐	☐
9	☐	☐	☐	☐	19	☐	☐	☐	☐	29	☐	☐	☐	☐	39	☐	☐	☐	☐
10	☐	☐	☐	☐	20	☐	☐	☐	☐	30	☐	☐	☐	☐	40	☐	☐	☐	☐

Examiner use: Correct answers _____ / 40 MCQ marks _____ / 80

A suitable patient has selected facial K-PDRN Reset using the current Venus microneedling-assisted protocol. Write four essential steps or controls from consultation through completion.

Examiner score: _____ / 4 Examiner initials: _____

Section B - Question 42: Patient selection - lip booster after filler with herpes symptoms

A patient requests K-Lip Dew ten days after lip filler. She has persistent swelling, a migrated lump and new tingling at the lip border. She also expects the booster to add volume and make the lips permanently pink. State four elements of your decision and counselling.

Examiner score: _____ / 4 Examiner initials: _____

Section B - Question 43: Product preparation and claims - K-Exo Glow

The clinic has received a lyophilized exosome skin product with a supplied diluent. Write four points covering verification, preparation, delivery and patient counselling before K-Exo Glow.

Examiner score: _____ / 4 Examiner initials: _____

During an under-eye PN treatment, the patient develops severe swelling, eye pain and blurred vision. State the first four actions or decisions required.

Examiner score: _____ / 4 Examiner initials: _____

Section B - Question 45: Integrated planning - acne-scar and recovery programme

A patient has completed control of active acne and is starting a Venus Viva/RF-microneedling scar programme. She asks for K-PDRN Reset, K-Colla Bounce and K-Exo Glow in the same visit because she wants faster results. Propose a safe four-point plan.

Examiner score: _____ / 4 Examiner initials: _____

Section C - Advanced Distinction answer grid

Question	A	B	C	D
46	[]	[]	[]	[]
47	[]	[]	[]	[]
48	[]	[]	[]	[]
49	[]	[]	[]	[]
50	[]	[]	[]	[]

Advanced Knowledge Score: _____ / 5

Candidate submission declaration

I confirm that I completed this assessment independently and did not use unauthorized notes, devices, applications, AI tools or assistance. I confirm that this answer sheet represents my own work.

Candidate signature	_____	Date	_____
Examiner signature	_____	Date	_____

Venus K-Glow Boosters

Examiner Answer Key, Rationales and Marking Guide | Version B

Assessment code	VA-KGLOW-B-2026.08
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Scoring and result policy

Section	Scoring	Maximum
A - MCQs	2 marks for each correct answer; no negative marking	80
B - Written	Four predefined marking points per question	20
Core total	MCQ + written	100
Advanced	1 point each; recorded separately	5

Result categories: 90% or above plus at least 4/5 advanced = Pass with Distinction; 80% or above without distinction criteria = Pass; 70-79% = Retest Required after targeted review; below 70% = Retraining Required + Retest; incomplete or invalid work = Invalid Attempt.

Source-control rule used to construct this assessment

The uploaded 99-slide training presentation is the primary source for scored core questions. The English transcript is used to capture verbal teaching where it is clear and consistent. Software-transcription errors and statements that conflict with the final slide content are not tested as core facts. Version B follows the same blueprint and difficulty standard as Version A but uses different questions, scenarios and written cases. Advanced Distinction questions use external authoritative sources and are scored separately.

Items intentionally not tested as exact core facts because the source material contains internal variation: exact Profhilo tissue-plane wording; the 1 cm versus 1.5 cm verbal tragus distance; exact per-papule K-Lumi volume; whether every eye/lip/face booster combination should occur on the same day; and whether every 2.5 mL lip syringe must always be deposited entirely within the lips. These should be standardized in the clinical SOP before being added to future versions.

Section A - MCQ answer key and teaching rationales

Q	Ans	Q	Ans	Q	Ans	Q	Ans
1	C	11	A	21	B	31	B
2	B	12	A	22	B	32	C
3	C	13	A	23	B	33	B
4	C	14	B	24	A	34	A
5	C	15	B	25	A	35	B
6	C	16	B	26	A	36	C
7	C	17	C	27	B	37	C
8	C	18	A	28	B	38	A
9	C	19	B	29	B	39	B
10	C	20	B	30	A	40	B

1. A patient requests K-Lumi Eyes because of dark circles. Examination shows mainly epidermal pigmentation, with no crepiness, dehydration, thin skin or fine lines. What is the best response?

- A. Proceed with K-Lumi because every type of dark circle responds to PN.
- B. Use a higher-volume K-Lumi treatment to compensate for the pigmentary cause.
- C. Explain that pigment is the dominant concern and direct the patient to a pigment-targeted plan; consider K-Lumi only if a separate under-eye skin-quality indication is present.
- D. Replace K-Lumi with K-Glass BAP injections under the eyes.

Correct answer: C

Rationale: K-Lumi is intended for selected under-eye skin-quality concerns such as fine lines, crepiness, dehydration and thin skin. Dark circles must be classified; pigment-dominant darkness needs a pigment-targeted direction rather than automatic booster treatment.

Why A is incorrect: The training specifically states that not every dark circle is suitable.

Why B is incorrect: More product does not correct an incorrect diagnosis.

Why D is incorrect: The facial BAP protocol is not an under-eye pigmentation treatment.

Domain: Indications & patient selection **Difficulty:** Hard **Type:** Applied

Source: Training Presentation slides 12 and 61-65; transcript pp. 14-16.

2. A patient has strong dynamic glabellar lines and also complains of oily skin and enlarged pores. Which plan best follows the Venus framework?

- A. Use K-Tox Refine alone and promise the same muscle-freezing result as conventional Botox.
- B. Assess conventional neuromodulator treatment for the dynamic lines and consider K-Tox Refine separately for pore, sebum and surface-quality concerns.
- C. Use K-Glass Hydration because all facial lines are caused by dehydration.
- D. Use K-Colla Bounce as an immediate substitute for both Botox and pore treatment.

Correct answer: B

Rationale: The dominant movement-related concern requires neuromodulator assessment. K-Tox Refine is a superficial refinement category for pores, sebum and fine surface lines and is not a substitute for standard Botox.

Why A is incorrect: The taught K-Tox product is not intended to produce clinically significant muscle relaxation.

Why C is incorrect: Hydration does not replace treatment of dynamic muscle activity.

Why D is incorrect: Collagen support is progressive and does not replace either indication.

Domain: Indications & patient selection **Difficulty:** Hard **Type:** Applied

Source: Training Presentation slides 12 and 78-83.

3. A doctor remembers a product from an older training slide, but it is not listed on the current clinic price sheet. What should happen before it is prescribed?

- A. Prescribe it because any product previously mentioned remains permanently approved.
- B. Use it only for staff members until new stock arrives.
- C. Verify the current Venus-approved product list and protocol with the current price sheet or management before prescribing or using it.
- D. Write the old product name but substitute whichever product is available at treatment time.

Correct answer: C

Rationale: The Venus treatment category remains constant, but product options may change. Doctors must verify current availability and approval and document the actual product used.

Why A is incorrect: Past inclusion does not guarantee current approval.

Why B is incorrect: Staff status does not remove product-governance requirements.

Why D is incorrect: Substitution without verification and documentation is unacceptable.

Domain: Post-care, consent & documentation **Difficulty:** Moderate **Type:** Applied

Source: Training Presentation slides 8-9 and all current-product tables; transcript pp. 1-2.

4. A patient had an aggressive peel four days ago and still has diffuse erythema, tightness and flaking. She requests a microneedling-assisted K-Glow booster today. What is the best decision?

- A. Proceed at maximum depth because the peel has already opened the skin.
- B. Proceed only on visibly dry areas.
- C. Delay treatment until the skin has recovered, because a recent aggressive procedure and active irritation increase risk.
- D. Change to an injectable booster so the epidermis is bypassed.

Correct answer: C

Rationale: Recent aggressive laser, peel or RF treatment and ongoing irritation are reasons to delay. Changing route does not make inflamed or recovering skin automatically suitable.

Why A is incorrect: This compounds trauma and increases inflammation and PIH risk.

Why B is incorrect: Partial treatment does not remove the underlying contraindication.

Why D is incorrect: An injectable route does not cancel the need for healthy, suitable tissue.

Domain: Contraindications & precautions **Difficulty:** Moderate **Type:** Applied

Source: Training Presentation slides 11-13 and 91; transcript pp. 2 and 21.

5. During a K-Lip Dew consultation, the patient reports new tingling at the lip border and has a history of recurrent cold sores. What should the doctor do?

- A. Proceed because the booster contains hydrating HA.
- B. Proceed after applying more numbing cream.
- C. Delay the treatment, assess for an evolving herpes outbreak and manage or obtain appropriate medical advice before reconsidering.
- D. Inject only the central lip and avoid the tingling area.

Correct answer: C

Rationale: An active or suspected herpes outbreak is a reason to delay lip treatment. Avoiding one area or increasing anaesthesia does not remove the risk of provoking or worsening an outbreak.

Why A is incorrect: Hydration benefit does not override infection risk.

Why B is incorrect: Numbing cream does not treat herpes.

Why D is incorrect: Partial injection still traumatizes the lip and is not an adequate safety strategy.

Domain: Contraindications & precautions **Difficulty:** Moderate **Type:** Applied

Source: Training Presentation slides 13 and 70; post-care slide 91.

6. A patient with poorly controlled diabetes is taking immunosuppressive medication and requests K-Exo Glow after seeing an online advertisement. What is the most appropriate approach?

- A. Proceed because exosomes are cell-free and therefore cannot cause complications.
- B. Proceed only with a lower Dermapen speed.
- C. Delay treatment and obtain appropriate medical and senior review because uncontrolled systemic disease and immunosuppression increase procedural risk.
- D. Use K-PDRN instead because it is derived from DNA.

Correct answer: C

Rationale: Uncontrolled diabetes, severe systemic illness and immunosuppression require avoidance or delay unless appropriately reviewed. No booster category automatically neutralizes these risks.

Why A is incorrect: Cell-free does not mean risk-free.

Why B is incorrect: Changing a device setting does not correct systemic unsuitability.

Why D is incorrect: Changing product category does not remove the medical risk.

Domain: Contraindications & precautions **Difficulty:** Hard **Type:** Applied

Source: Training Presentation slide 13.

7. A supplier representative says an exosome preparation may be injected, but the product documentation available in the clinic only supports post-procedure topical use. What should the doctor do?

- A. Inject it because the supplier verbally confirmed the route.
- B. Inject a small test dose first.
- C. Do not inject it; use only a route supported by verified product documentation and the current Venus protocol.
- D. Mix it with saline so the route becomes acceptable.

Correct answer: C

Rationale: Route selection must follow verified IFU or intended-use documentation and the Venus protocol. Verbal claims, test dosing or dilution do not authorize injection.

Why A is incorrect: Supplier conversation is not a substitute for verified product instructions.

Why B is incorrect: A test dose does not validate an unapproved route.

Why D is incorrect: Dilution does not create route approval.

Domain: Treatment protocol & technique **Difficulty:** Hard **Type:** Applied

Source: Training Presentation slides 14 and 58; transcript pp. 2-3 and 13.

8. A sterile Dermapen cartridge is opened, attached and then accidentally touches a non-sterile trolley before the procedure begins. What is the correct action?

- A. Wipe the cartridge with alcohol and continue.
- B. Use it only on the forehead because that area is less sensitive.
- C. Discard it and attach a new sterile cartridge.
- D. Keep it for the same patient at the next session.

Correct answer: C

Rationale: Sterile needles, tips and cartridges must be maintained throughout the procedure. Once contaminated, the cartridge must be replaced rather than cleaned, repurposed or stored.

Why A is incorrect: Surface wiping does not restore validated sterility.

Why B is incorrect: Treatment area does not make contamination acceptable.

Why D is incorrect: A new sterile cartridge is required for each treatment session.

Domain: Complications & emergency management **Difficulty:** Moderate **Type:** Applied

Source: Training Presentation slides 14-17; transcript pp. 3-5.

9. An assistant hands the doctor an unlabeled prefilled syringe, stating that it is the product sold to the patient. What should the doctor do before treatment?

- A. Proceed if the syringe colour looks familiar.
- B. Ask the patient whether the product name sounds correct.
- C. Stop and verify the exact prescribed product, original packaging, integrity, batch, expiry and quantity before using it.
- D. Use half the syringe first and verify later if results are poor.

Correct answer: C

Rationale: The treating doctor must personally confirm that the product matches the prescription and verify packaging integrity, batch, expiry and quantity. Appearance or verbal reassurance is insufficient.

Why A is incorrect: Visual familiarity does not establish identity or traceability.

Why B is incorrect: The patient is not responsible for product verification.

Why D is incorrect: Treatment must not begin before verification.

Domain: Post-care, consent & documentation **Difficulty:** Hard **Type:** Applied

Source: Training Presentation slides 17 and 90; transcript pp. 5 and 15.

10. Three days after microneedling-assisted K-Colla Bounce, a patient reports increasing warmth, spreading redness, pus and fever. What is the correct interpretation?

- A. Normal collagen-induction downtime that requires reassurance only.
- B. Expected flaking that should be treated with a facial scrub.
- C. A possible infection requiring urgent clinical assessment and escalation.
- D. A normal reaction that proves the product reached the dermis.

Correct answer: C

Rationale: Pus, fever, spreading redness and increasing warmth are urgent red flags for infection, not routine post-procedure erythema or dryness.

Why A is incorrect: Expected reactions should improve rather than spread with systemic symptoms.

Why B is incorrect: Scrubbing can worsen irritation and infection.

Why D is incorrect: Complication signs do not demonstrate treatment efficacy.

Domain: Complications & emergency management **Difficulty:** Moderate **Type:** Applied

Source: Training Presentation slides 18 and 91.

11. Why does a non-cross-linked HA skin booster generally behave differently from a cross-linked HA filler?

- A. Non-cross-linked HA is designed mainly for water binding and tissue-quality improvement, while cross-linking creates a more degradation-resistant gel suited to filling and structural support.
- B. Non-cross-linked HA permanently freezes muscle movement.
- C. Cross-linked HA is always topical, while non-cross-linked HA is always injectable.
- D. There is no clinically relevant difference once HA enters the skin.

Correct answer: A

Rationale: The training contrasts fluid non-cross-linked HA used mainly for hydration and skin quality with more stable cross-linked HA gels used for longer-lasting filling, support and volume.

Why B is incorrect: HA does not act as a neuromodulator.

Why C is incorrect: Cross-linking does not define topical versus injectable use.

Why D is incorrect: Formulation materially affects rheology and clinical purpose.

Domain: Scientific foundation & mechanism **Difficulty:** Hard **Type:** Concept

Source: Training Presentation slides 22-24.

12. Which K-Glass product description is fully correct according to the training table?

- A. Profhilo: 64 mg non-cross-linked HA in 2 mL; Lumi Pro: 40 mg non-cross-linked HA in 2 mL; Lamist: 68 mg non-cross-linked HA plus succinic acid in 2 mL.
- B. Profhilo: 20 mg PN in 2 mL; Lumi Pro: 64 mg cross-linked HA; Lamist: recombinant collagen only.
- C. Profhilo: 40 mg cross-linked HA; Lumi Pro: 68 mg HA plus CMC; Lamist: 2% PDRN.
- D. All three products have identical composition and differ only by country of origin.

Correct answer: A

Rationale: The presentation lists Profhilo 64 mg/2 mL, Lumi Pro 40 mg/2 mL, and Lamist 68 mg non-cross-linked HA plus succinic acid/2 mL.

Why B is incorrect: This mixes eye/PN and collagen categories.

Why C is incorrect: The concentrations and ingredients are mismatched.

Why D is incorrect: The products differ in formulation and clinical positioning.

Domain: Scientific foundation & mechanism **Difficulty:** Hard **Type:** Product-specific

Source: Training Presentation slide 22.

13. A patient has marked dehydration plus early loss of elasticity and wants the premium K-Glass option that has a broader tissue-quality objective. Which product best matches the taught selection logic?

- A. Profhilo, because it is positioned for hydration plus bioremodelling and tissue-quality improvement.
- B. Lumi Meso Tox, because pores and sebum determine elasticity.
- C. Lapuroon Vivid, because all DNA-derived products act as HA.
- D. Baccio Lip Booster, because vitamin B12 stimulates facial volume.

Correct answer: A

Rationale: Profhilo is the premium K-Glass option for hydration with a broader bioremodelling objective, including elasticity and tissue quality. The other choices belong to different categories and indications.

Why B is incorrect: K-Tox is a surface-refinement category.

Why C is incorrect: PDRN is not an HA hydration product.

Why D is incorrect: Baccio is a lip-quality product, not a facial bioremodeller.

Domain: Indications & patient selection **Difficulty:** Hard **Type:** Applied

Source: Training Presentation slides 20-24.

14. How is the nasal-base BAP point located in the marking guide?

- A. At the midpoint of the nasolabial fold, independent of other landmarks.
- B. At the intersection of a line from the nostril toward the tragus and a vertical line from the pupil.
- C. Two centimetres directly below the medial canthus.
- D. At the lateral corner of the mouth.

Correct answer: B

Rationale: The nasal-base point is marked where the nostril-to-tragus line intersects the vertical mid-pupillary line.

Why A is incorrect: The guide uses defined intersecting landmarks.

Why C is incorrect: This is not the taught measurement.

Why D is incorrect: The oral commissure is not the nasal-base BAP point.

Domain: Treatment protocol & technique **Difficulty:** Hard **Type:** Applied anatomy

Source: Training Presentation slides 26-29; transcript pp. 6-7.

15. Which method best reflects the taught chin BAP marking approach?

- A. Mark the centre of the lower lip and inject directly on the midline.
- B. Create a chin midline, identify the upper third, then mark approximately 1.5 cm laterally from the relevant intersection on each side.
- C. Mark directly over the mental foramen without palpation.
- D. Use the mandibular angle point for the chin as well.

Correct answer: B

Rationale: The guide uses the central chin line, the upper-third intersection and a lateral measurement of approximately 1.5 cm to locate the bilateral chin points.

Why A is incorrect: The injection points are not directly on the midline.

Why C is incorrect: The guide is intended to avoid unsafe or inappropriate placement.

Why D is incorrect: The chin and mandibular-angle points are separate landmarks.

Domain: Treatment protocol & technique **Difficulty:** Hard **Type:** Applied anatomy

Source: Training Presentation slides 26-29; transcript pp. 6-7.

16. Which sequence is most consistent with the Venus K-Glass BAP workflow?

- A. Open the syringe, inject by visual estimation, then mark any missed points.
- B. Mark the ten points first, verify the product, attach and prime the needle, orient the bevel appropriately, inject the complete 2 mL slowly in controlled 0.2 mL deposits and observe the skin.
- C. Dilute the syringe with saline, use twenty points and store any remainder.
- D. Inject one side only and use the second half at the next visit.

Correct answer: B

Rationale: The taught workflow requires marking before opening or using the syringe, product verification, priming, controlled technique, 0.2 mL at ten points and use of the complete 2 mL syringe for one patient.

Why A is incorrect: Marking after injection defeats the standardized protocol.

Why C is incorrect: Dilution and remainder storage are not part of the protocol.

Why D is incorrect: Half-syringe treatment and storage are not permitted.

Domain: Treatment protocol & technique **Difficulty:** Hard **Type:** Applied

Source: Training Presentation slides 25-26 and 33; transcript pp. 6 and 9.

17. Before starting a K-Glass HA injection, the doctor discovers that the treatment room has no accessible hyaluronidase and the emergency escalation contact cannot be reached. What should happen?

- A. Proceed because vascular compromise is uncommon with non-cross-linked HA.
- B. Proceed with half the dose and arrange the emergency supplies later.
- C. Postpone the injectable treatment until the required emergency medication, supportive equipment and escalation pathway are available.
- D. Replace hyaluronidase with a cold compress.

Correct answer: C

Rationale: HA injection should not begin without emergency readiness. Hyaluronidase, supportive measures and a clear senior escalation pathway must be accessible before treatment, even when the complication is uncommon.

Why A is incorrect: Low frequency does not justify removing emergency preparedness.

Why B is incorrect: Reducing dose does not make an unprepared procedure acceptable.

Why D is incorrect: A cold compress is not a substitute and is contrary to the taught vascular protocol.

Domain: Complications & emergency management **Difficulty:** Hard **Type:** Applied emergency

Source: Training Presentation slides 14, 31-33; transcript pp. 8-10.

18. Immediately after a BAP deposit, a small soft papule forms without severe pain, colour change or visual symptoms. What is the best interpretation?

- A. This alone is an expected temporary reaction that should settle; continue monitoring and counsel the patient.
- B. It proves arterial occlusion and requires hospital transfer.
- C. It means the complete syringe must be dissolved.
- D. It shows the product was injected as a filler and must be massaged aggressively.

Correct answer: A

Rationale: Small injection papules or bumps are expected after K-Glass BAP injections and usually settle. Severe pain, blanching, livedo, delayed refill or visual symptoms would change the response to an emergency pathway.

Why B is incorrect: A papule without vascular red flags is not by itself an occlusion.

Why C is incorrect: Routine papules do not require dissolution.

Why D is incorrect: Aggressive massage is not the routine response described.

Domain: Complications & emergency management **Difficulty:** Hard **Type:** Applied

Source: Training Presentation slides 18 and 33.

19. After a Dermapen-assisted booster, which advice is most consistent with the universal Venus post-care?

- A. Resume retinol and exfoliating acids the same evening to enhance penetration.
- B. Avoid touching and makeup initially, avoid heat and strenuous exercise for about 24-48 hours, use gentle skincare and sunscreen, avoid strong actives for several days and other irritating procedures for roughly 1-2 weeks unless advised.
- C. Scrub the skin daily to remove flaking.
- D. Book an aggressive peel within 48 hours while microchannels remain open.

Correct answer: B

Rationale: The post-care protects the healing barrier and reduces infection, irritation and PIH risk. Early actives, scrubbing, heat and aggressive procedures are avoided.

Why A is incorrect: Strong actives can worsen irritation.

Why C is incorrect: Scrubbing traumatizes recovering skin.

Why D is incorrect: An early aggressive procedure compounds inflammation.

Domain: Post-care, consent & documentation **Difficulty:** Hard **Type:** Applied post-care

Source: Training Presentation slides 91 and 96; transcript p. 21.

20. In the K-Colla Bounce name, what does "bounce" describe?

- A. Immediate structural volume and cheek projection.
- B. Improved elasticity, firmness and the skin's ability to recover its shape or resilience.
- C. Temporary oedema after treatment.
- D. Complete removal of acne scars.

Correct answer: B

Rationale: Bounce refers to dermal elasticity, firmness and resilience, not filler-like projection or temporary swelling.

Why A is incorrect: K-Colla is a skin-quality category, not a volumizing filler.

Why C is incorrect: Oedema is not the treatment objective.

Why D is incorrect: The training states that it is not a scar-specific replacement.

Domain: Scientific foundation & mechanism **Difficulty:** Moderate **Type:** Concept

Source: Training Presentation slides 35-40; transcript p. 10.

21. Which product-route pairing is correct under the current K-Colla training?

- A. SeduSkin Hylagen3 - inject by BAP; Rejucoll - intramuscular; Karisma - Dermapen only.
- B. SeduSkin Hylagen3 - Venus microneedling protocol and do not inject; Rejucoll - current Venus microneedling protocol despite manufacturer injection options; Karisma - product-specific injectable technique, not routine microneedling.
- C. All collagen products use the same technique because the ingredient category determines the route.
- D. All three products are topical masks.

Correct answer: B

Rationale: The training emphasizes product-specific routes. SeduSkin is not injected; Rejucoll is currently microneedled at Venus; Karisma has its own injectable technique and is not treated as a generic microneedling product.

Why A is incorrect: Each route is mismatched.

Why C is incorrect: A collagen ingredient does not automatically determine route.

Why D is incorrect: These are professional treatment products, not masks.

Domain: Treatment protocol & technique **Difficulty:** Hard **Type:** Product-specific

Source: Training Presentation slides 37-39; transcript pp. 10-11.

22. A doctor is using a 2.5 mL Rejucoll syringe through the current Venus microneedling protocol. Which product-management approach best matches the verbal training?

- A. Use the full syringe on the first cheek and substitute saline on the second.
- B. Plan approximately equal distribution, around 1.25 mL per side, while observing coverage and the approved endpoint.
- C. Use 0.5 mL and save the rest for other patients.
- D. Mix it with PRP to increase the volume to 5 mL.

Correct answer: B

Rationale: The verbal training highlights even planning of the limited 2.5 mL volume, approximately 1.25 mL per side, while maintaining appropriate microneedling coverage. Sharing, storage and unapproved mixing are not allowed.

Why A is incorrect: This produces uneven treatment and inappropriate substitution.

Why C is incorrect: Opened-product sharing or storage is not supported.

Why D is incorrect: Unapproved mixing changes the treatment and sterility profile.

Domain: Treatment protocol & technique **Difficulty:** Hard **Type:** Applied

Source: Training Presentation slide 37; transcript p. 10.

23. A patient with a darker skin type and a strong history of post-inflammatory hyperpigmentation is suitable for K-Colla microneedling. How should the procedure be modified?

- A. Use more passes and deeper settings to overcome melanin activity.
- B. Use conservative depth and passes, maintain controlled erythema rather than aggressive bleeding, and reinforce post-care and sun protection.
- C. Create uniform bleeding because this prevents PIH.
- D. Combine an aggressive peel in the same visit.

Correct answer: B

Rationale: The training advises conservative settings in darker or PIH-prone skin and avoids over-treatment. Controlled erythema, careful technique and post-care reduce inflammatory risk.

Why A is incorrect: More trauma can increase PIH risk.

Why C is incorrect: Bleeding is not protective against pigmentation.

Why D is incorrect: Stacking inflammatory procedures increases risk.

Domain: Contraindications & precautions **Difficulty:** Hard **Type:** Applied precaution

Source: Training Presentation slides 39-40.

24. Which comparison between PN and PDRN is most consistent with the training?

- A. PN generally has longer DNA chains and a more viscoelastic support emphasis; PDRN contains shorter fragments and has a lighter repair/recovery emphasis.
- B. PN is cross-linked HA, while PDRN is collagen.
- C. PN and PDRN are identical terms with no difference in chain length or clinical emphasis.
- D. PDRN is a live stem-cell preparation and PN is fish oil.

Correct answer: A

Rationale: The presentation differentiates longer-chain, more viscoelastic PN from shorter PDRN fragments with a repair and recovery emphasis.

Why B is incorrect: Neither is HA or collagen.

Why C is incorrect: The training explicitly distinguishes chain length and tissue behaviour.

Why D is incorrect: Neither product is stem cells or fish oil.

Domain: Scientific foundation & mechanism **Difficulty:** Hard **Type:** Concept

Source: Training Presentation slide 43.

25. Which product profile is correctly matched in the K-PDRN Reset table?

- A. Rejuran Healer: 2 mL syringe with 2% PN; Lapuroon Aurora Super: 2.5 mL syringe with 2% PDRN; Lapuroon Aurora Vivid: 5 mL vial with 0.2% PDRN.
- B. Rejuran Healer: 5 mL exosome vial; Lapuroon Super: cross-linked HA filler; Vivid: botulinum toxin.
- C. All three contain the same volume and concentration.
- D. Rejuran is a lip booster, Super is an eye filler and Vivid is a collagen implant.

Correct answer: A

Rationale: The training identifies Rejuran as a premium 2 mL 2% PN option, Lapuroon Super as 2.5 mL 2% PDRN and Vivid as a 5 mL 0.2% PDRN vial.

Why B is incorrect: The ingredients and categories are incorrect.

Why C is incorrect: The products differ in volume, composition and positioning.

Why D is incorrect: All three are misclassified.

Domain: Scientific foundation & mechanism **Difficulty:** Hard **Type:** Product-specific

Source: Training Presentation slide 45; transcript pp. 11-12.

26. During K-PDRN Reset, an assistant proposes holding the syringe and dropping product while the doctor uses the Dermapen. What does the verbal training prefer?

- A.** The doctor should control both product droplets and microneedling delivery so distribution and remaining volume can be monitored directly.
- B.** The assistant should inject the product while the doctor microneedles.
- C.** The product should be poured over the full face before cleansing.
- D.** The product should be mixed into numbing cream.

Correct answer: A

Rationale: The verbal demonstration emphasized a controlled droplet approach in which the doctor manages the syringe and product distribution while performing microneedling.

Why B is incorrect: Injection is not the current facial Venus PDRN booster route.

Why C is incorrect: Product should be applied in a controlled sterile treatment sequence.

Why D is incorrect: Mixing into anaesthetic is not an approved delivery method.

Domain: Treatment protocol & technique **Difficulty:** Hard **Type:** Applied

Source: Training Presentation slides 47-50; transcript p. 12.

27. Which K-PDRN plan follows the taught Venus protocol?

- A.** Mix PDRN with PRP and saline in one bowl to improve spread.
- B.** Use controlled Dermapen-assisted delivery, or topical application after RF microneedling where protocol allows, without casual mixing with PRP, saline or other boosters.
- C.** Inject all facial PDRN products by the BAP technique.
- D.** Use aggressive bleeding as the endpoint.

Correct answer: B

Rationale: Current facial K-PDRN delivery is microneedling-assisted, with possible post-RF topical use. Unapproved mixing and aggressive needling are specifically discouraged.

Why A is incorrect: The training says not to mix unless an approved protocol allows.

Why C is incorrect: The current Venus facial protocol is not generic BAP injection.

Why D is incorrect: Controlled erythema is the intended endpoint.

Domain: Treatment protocol & technique **Difficulty:** Hard **Type:** Applied

Source: Training Presentation slides 47-50.

28. A patient with deep tethered acne scars asks whether K-PDRN Reset alone will cure the scars. What is the best counselling?

- A. Promise complete scar removal after one session.
- B. Explain that K-PDRN may support skin quality and recovery but is not a cure for deep scars; scar-specific treatment such as RF microneedling, subcision or laser may be the primary plan.
- C. Use a higher PDRN concentration and avoid scar-specific treatment.
- D. Replace PDRN with K-Lip Dew on the scars.

Correct answer: B

Rationale: K-PDRN supports repair, vitality and post-acne skin quality but does not replace scar-specific procedures or cure deep scars.

Why A is incorrect: This is an unrealistic and unsupported promise.

Why C is incorrect: Concentration does not correct the mismatch in treatment indication.

Why D is incorrect: K-Lip Dew is a lip-quality category.

Domain: Indications & patient selection **Difficulty:** Hard **Type:** Applied

Source: Training Presentation slides 12 and 42-47.

29. Which statement best describes the role of exosomes in the K-Exo module?

- A. They are living stem cells that permanently replace damaged skin cells.
- B. They are extracellular signalling vesicles carrying bioactive cargo; they support cell-to-cell communication but do not become new cells.
- C. They are cross-linked HA particles used for volume.
- D. They are a form of injectable Botox.

Correct answer: B

Rationale: The training presents exosomes as cell-free extracellular messengers carrying proteins, lipids, growth factors and RNA. They support signalling and recovery rather than becoming new cells.

Why A is incorrect: Calling the treatment stem-cell therapy is specifically prohibited.

Why C is incorrect: Exosomes are not HA filler.

Why D is incorrect: They are unrelated to neuromuscular blockade.

Domain: Scientific foundation & mechanism **Difficulty:** Moderate **Type:** Concept

Source: Training Presentation slides 52-54 and 59.

30. Which comparison of the two K-Exo product options is correct according to the training?

- A. Exoxe: made in Korea, 6 billion exosomes, described as amniotic-fluid stem-cell derived; ASCE+ ExCoBio: made in Korea, 5 billion exosomes, described as adipose-tissue derived.
- B. Exoxe: 5 billion adipose-derived; ASCE+: 6 billion salmon-derived.
- C. Both are PN injections from salmon sperm cells.
- D. Both are cross-linked HA fillers supplied in 2 mL syringes.

Correct answer: A

Rationale: The presentation lists Exoxe as a 6-billion amniotic-fluid-derived option and ASCE+ as a 5-billion adipose-tissue-derived option, both made in Korea.

Why B is incorrect: The counts and sources are reversed and incorrect.

Why C is incorrect: These are exosome preparations, not PN products.

Why D is incorrect: They are not HA fillers.

Domain: Scientific foundation & mechanism **Difficulty:** Hard **Type:** Product-specific

Source: Training Presentation slide 56; transcript pp. 13-14.

31. What is the taught preparation and skin-delivery approach for the lyophilized K-Exo products?

- A. Inject the dry powder directly before adding diluent.
- B. Reconstitute the lyophilized vial with the supplied diluent for the skin preparation, then use the verified Dermapen-assisted or post-RF topical protocol; inject only if explicitly supported.
- C. Mix the powder with a strong acid peel.
- D. Use normal saline for every skin and hair protocol without checking product instructions.

Correct answer: B

Rationale: For the skin protocol described, the lyophilized product is reconstituted with the supplied diluent and delivered by the approved microneedling or post-procedure route. Injection requires explicit support.

Why A is incorrect: Dry powder is not administered directly.

Why C is incorrect: Strong irritants should not be mixed with exosome preparations.

Why D is incorrect: The training notes that skin and hair preparation can differ.

Domain: Treatment protocol & technique **Difficulty:** Hard **Type:** Applied

Source: Training Presentation slides 56-58; transcript p. 13.

32. Which statement is most appropriate when counselling and documenting K-Exo Glow?

- A. Guarantee DNA repair and reversal of cellular ageing because the vial has billions of particles.
- B. Call it stem-cell therapy to simplify the explanation.
- C. Explain that it is a signalling and recovery-support treatment with variable evidence and product quality; verify source, preparation, storage, batch and technique and avoid disease-cure claims.
- D. State that particle count alone proves clinical superiority.

Correct answer: C

Rationale: The training emphasizes realistic claims, variable product quality and the importance of source, manufacturing, storage, preparation and documentation. Stem-cell and cure claims are prohibited.

Why A is incorrect: The training rejects DNA-repair and guaranteed anti-ageing claims.

Why B is incorrect: Exosomes are cell-free and should not be presented as stem-cell therapy.

Why D is incorrect: Count alone does not establish quality or efficacy.

Domain: Post-care, consent & documentation **Difficulty:** Hard **Type:** Applied policy

Source: Training Presentation slides 54-59.

33. Which patient is the strongest candidate for K-Lumi Eyes?

- A. A patient with severe fat prolapse and large eye bags requesting their removal.
- B. A patient with thin, mildly dehydrated, crepey under-eye skin and fine superficial lines, without major bags or unresolved swelling.
- C. A patient with active periocular dermatitis.
- D. A patient with major tear-trough structural hollowing who refuses filler assessment.

Correct answer: B

Rationale: K-Lumi is best suited to selected under-eye skin-quality concerns. Severe bags, major structural hollowing, infection, dermatitis and unresolved swelling require a different direction or delay.

Why A is incorrect: Boosters do not remove severe eye bags.

Why C is incorrect: Active dermatitis is a reason to delay.

Why D is incorrect: A structural hollow requires appropriate structural assessment.

Domain: Indications & patient selection **Difficulty:** Hard **Type:** Applied diagnosis

Source: Training Presentation slides 61 and 64.

34. Which shortlisted under-eye product pairing is correct in the training?

- A. Lumi Pro Eyes: 5 mg in a 1 mL syringe; AMI Eyes: 20 mg in a 2 mL syringe.
- B. Lumi Pro Eyes: 64 mg HA in 2 mL; AMI Eyes: 5 billion exosomes.
- C. Lumi Pro Eyes: 5 mL mesotox vial; AMI Eyes: 2.5 mL lip booster.
- D. Both are cross-linked volumizing fillers.

Correct answer: A

Rationale: The training lists Lumi Pro Eyes as 5 mg/1 mL and AMI Eyes as 20 mg/2 mL, both within the PN-based periocular category.

Why B is incorrect: These values belong to other categories.

Why C is incorrect: The products are misclassified.

Why D is incorrect: They are under-eye skin-quality boosters, not volumizing fillers.

Domain: Treatment protocol & technique **Difficulty:** Hard **Type:** Product-specific

Source: Training Presentation slide 62; transcript pp. 14-15.

35. Which technique description best matches the taught K-Lumi injection principles?

- A. Deep orbital boluses using the full 4 mm needle perpendicular to the skin.
- B. Superficial papillary-dermal microinjections using conservative volume, approximately a 10-20 degree angle and the approved 32G x 4 mm mesotherapy needle, while avoiding deep orbital placement and the lash line.
- C. Intramuscular injection into the orbicularis to freeze movement.
- D. BAP injection at the five facial points.

Correct answer: B

Rationale: The periocular protocol is conservative and superficial, using product-specific microinjections and avoiding deep orbital placement, large boluses and unsupported lash-line injection.

Why A is incorrect: This is unsafe and contrary to the taught superficial approach.

Why C is incorrect: K-Lumi is not a standard Botox treatment.

Why D is incorrect: The facial BAP map is a different protocol.

Domain: Treatment protocol & technique **Difficulty:** Hard **Type:** Applied technique

Source: Training Presentation slide 65; transcript pp. 15-16.

36. After an under-eye PN injection, a patient develops severe swelling, eye pain and blurred vision. What is the best response?

- A. Reassure the patient that all under-eye puffiness is normal.
- B. Inject hyaluronidase and discharge immediately because it dissolves PN.
- C. Treat this as an urgent eye-related complication, stop treatment, assess and escalate for immediate medical or ophthalmic review; remember that hyaluronidase does not dissolve pure PN/PDRN.
- D. Massage the orbit firmly until vision returns.

Correct answer: C

Rationale: Eye pain, visual symptoms and severe swelling are urgent red flags. Pure PN/PDRN is not dissolved by hyaluronidase, so emergency assessment and appropriate referral must not be delayed.

Why A is incorrect: The symptom combination is not routine puffiness.

Why B is incorrect: Hyaluronidase does not reverse pure PN/PDRN and discharge is unsafe.

Why D is incorrect: Forceful massage is not an appropriate response to visual symptoms.

Domain: Complications & emergency management **Difficulty:** Hard **Type:** Applied emergency

Source: Training Presentation slide 65.

37. Which finding is the clearest reason to delay K-Lip Dew?

- A. Dry lips with fine surface lines and no active disease.
- B. A settled lip filler placed three months ago with healthy tissue.
- C. Active cheilitis with fissuring and a possible dental infection.
- D. A patient who wants hydration rather than volume.

Correct answer: C

Rationale: Active inflammation, cheilitis, infection and dental infection require assessment and delay. The other choices can be compatible with treatment after appropriate review.

Why A is incorrect: This is a typical K-Lip indication.

Why B is incorrect: Settled filler does not automatically exclude a booster.

Why D is incorrect: Hydration without volume is the intended objective.

Domain: Contraindications & precautions **Difficulty:** Moderate **Type:** Applied

Source: Training Presentation slides 70 and 76.

38. Which K-Lip Dew statement is correct according to the training?

- A.** Skinfill Baccio is a 2.5 mL non-cross-linked HA 20 mg/mL plus vitamin B12 lip booster used with the approved stamp needle; Lumi Pro Lips is a 1 mL 22 mg/mL non-cross-linked HA option that can use the same approved pink stamp-needle approach.
- B.** Baccio is a cross-linked Russian-lip filler and Lumi Pro Lips is injectable Botox.
- C.** Both products must be shared between patients because the lip requires only 0.2 mL.
- D.** The pink colour guarantees permanent pink pigmentation of the lips.

Correct answer: A

Rationale: The training identifies Baccio and Lumi Pro Lips as non-cross-linked HA lip-quality boosters, not fillers. The current Venus delivery uses the product-specific approved stamp-needle approach.

Why B is incorrect: Neither product is standard filler or Botox.

Why C is incorrect: Syringe sharing is prohibited.

Why D is incorrect: The purpose is hydration and quality, not guaranteed pigmentation.

Domain: Treatment protocol & technique **Difficulty:** Hard **Type:** Product-specific

Source: Training Presentation slides 71-75; transcript pp. 17-18.

39. A patient requests K-Lip Dew seven days after lip filler. Examination shows persistent swelling, migration and tender nodules. What is the best plan?

- A.** Proceed because a booster will smooth and dissolve the migration.
- B.** Delay the booster, assess and correct the filler-related problem first, and reconsider only after the lips are clinically healthy and settled.
- C.** Use more booster on the migrated side to restore symmetry.
- D.** Proceed if the patient signs that results are not guaranteed.

Correct answer: B

Rationale: Recent filler with unresolved swelling, migration, nodules or pain requires delay and appropriate filler assessment or correction. A booster does not dissolve or correct a problematic filler result.

Why A is incorrect: Lip booster is not a dissolving treatment.

Why C is incorrect: Adding product may worsen the problem.

Why D is incorrect: Consent does not make an unsuitable procedure appropriate.

Domain: Contraindications & precautions **Difficulty:** Hard **Type:** Applied

Source: Training Presentation slide 76; transcript p. 18.

40. A patient with oily skin and enlarged pores asks for K-Tox Refine and expects the forehead to be completely frozen. Which counselling and protocol statement is most accurate?

- A.** Lumi Meso Tox is injected intramuscularly exactly like standard Botox and guarantees muscle paralysis.
- B.** K-Tox Refine uses the complete 5 mL Lumi Meso Tox vial through the approved superficial microneedling protocol for pore, sebum and surface refinement; it is not a substitute for injectable Botulinum Toxin Type A, and possible problems include irritation, dryness, uneven refinement or eyebrow heaviness.
- C.** K-Tox is a collagen filler used for volume loss.
- D.** The vial should be divided among five patients to prevent heaviness.

Correct answer: B

Rationale: The current K-Tox product is a 5 mL microneedling preparation for superficial refinement, not clinically significant muscle relaxation. The training also lists possible adverse or unsatisfactory outcomes that require counselling.

Why A is incorrect: The route and objective are incorrect.

Why C is incorrect: K-Tox is not a filler.

Why D is incorrect: The complete vial is used for one client and sharing is unsafe.

Domain: Complications & emergency management **Difficulty:** Hard **Type:** Applied

Source: Training Presentation slides 78-83; transcript pp. 18-19.

Section B - Written-answer model responses and marking rubrics

Award 1 mark for each essential point. Accept clinically equivalent wording when the meaning is clear. Do not award marks for vague statements that omit the required decision or action. Maximum 4 marks per question.

41. Protocol reconstruction - K-PDRN Reset with Dermapen

A suitable patient has selected facial K-PDRN Reset using the current Venus microneedling-assisted protocol. Write four essential steps or controls from consultation through completion.

Mark	Essential point
1	Confirm the dominant indication, screen contraindications including active inflammation and fish/salmon/seafood or previous PN/PDRN reaction, obtain consent and standardized photographs.
1	Verify the exact approved product, packaging, batch, expiry and quantity; prepare sterile equipment and apply/remove anaesthesia and cleanse as required.
1	Use the doctor-controlled droplet and Dermapen technique with appropriate regional settings and controlled erythema, avoiding aggressive bleeding and unapproved mixing with PRP, saline or other boosters.
1	Document product, quantity, device settings/technique and reaction; give written aftercare and red-flag advice and plan the recommended course and review.

Source: Slides 13-18, 42-50 and 88-96; transcript pp. 11-12 and 20-21.

42. Patient selection - lip booster after filler with herpes symptoms

A patient requests K-Lip Dew ten days after lip filler. She has persistent swelling, a migrated lump and new tingling at the lip border. She also expects the booster to add volume and make the lips permanently pink. State four elements of your decision and counselling.

Mark	Essential point
1	Do not proceed: recent filler with unresolved swelling/migration and possible herpes symptoms makes the lips unsuitable at this visit.
1	Assess filler product/date/volume, pain, vascular or inflammatory signs, cold-sore history and possible dental or lip infection; arrange appropriate management or referral.
1	Explain that K-Lip Dew is for hydration, softness and fine lip lines, not major volume, filler correction, Russian-lip shaping or guaranteed permanent pink colour.
1	Document the consultation and reconsider only after the filler problem and possible herpes have been appropriately managed and the lips are clinically healthy and settled.

Source: Slides 70-76 and 91; transcript pp. 17-18.

43. Product preparation and claims - K-Exo Glow

The clinic has received a lyophilized exosome skin product with a supplied diluent. Write four points covering verification, preparation, delivery and patient counselling before K-Exo Glow.

Mark	Essential point
1	Verify the exact Venus-approved product, source/intended use, packaging integrity, storage status, batch and expiry before preparation.
1	Reconstitute the lyophilized vial with the supplied diluent according to product instructions; do not assume the hair-preparation method is identical to the skin method.
1	Use the current verified Dermapen-assisted or post-RF topical protocol; do not inject unless explicitly supported and do not mix with strong acids, irritants or unrelated boosters.
1	Counsel that exosomes support signalling and recovery but are not stem cells, do not become new cells and do not justify disease-cure, DNA-repair or guaranteed reversal claims; document product and technique.

Source: Slides 52-59; transcript pp. 12-14.

44. Urgent management - visual symptoms after K-Lumi Eyes

During an under-eye PN treatment, the patient develops severe swelling, eye pain and blurred vision. State the first four actions or decisions required.

Mark	Essential point
1	Stop the procedure immediately and keep the patient under direct medical supervision.
1	Assess and document the time of onset, vision symptoms, pain, swelling, colour and relevant vital or neurological findings; alert the senior doctor/Medical Director.
1	Arrange immediate urgent ophthalmic/emergency assessment or referral; do not delay because visual symptoms are an emergency.
1	Do not rely on hyaluronidase to dissolve pure PN/PDRN; document the product, volume, sites and all actions and hand over clearly to the receiving team.

Source: Slide 65; universal red-flag slide 18.

45. Integrated planning - acne-scar and recovery programme

A patient has completed control of active acne and is starting a Venus Viva/RF-microneedling scar programme. She asks for K-PDRN Reset, K-Colla Bounce and K-Exo Glow in the same visit because she wants faster results.

Propose a safe four-point plan.

Mark	Essential point
1	Reassess that active inflammation and infection are controlled and identify the separate objectives: scar-specific RF treatment versus repair/recovery, texture and collagen-support goals.
1	Do not automatically stack all boosters; use only same-session add-ons specifically supported by the relevant product and Venus protocol, otherwise stage treatments after the skin has settled.
1	Explain that K-PDRN/K-Exo may support recovery and skin quality and K-Colla may support progressive firmness/texture, but none replaces the scar-specific procedure or guarantees scar cure.
1	Create and document a staged course with consent, photographs, exact product/quantity/route/settings at each session, written post-care and adequate intervals based on inflammation and downtime.

Source: Slides 12, 39-40, 47-50, 58-59 and 85-96; transcript pp. 19-21.

Section C - Advanced Distinction answer key

46: B 47: B 48: A 49: A 50: A

46. What is the most evidence-based interpretation of a negative aspiration before an HA injection?

- A. It proves the needle is outside a blood vessel and makes a large bolus safe.
- B. It cannot reliably exclude intravascular placement; anatomical knowledge, small controlled doses, slow low-pressure injection and continuous observation remain essential.
- C. It eliminates the need for hyaluronidase availability.
- D. It is reliable only when the patient is fasting.

Correct answer: B

Rationale: Published analyses conclude that aspiration can produce false-negative results and should not be relied upon as a safety guarantee. Injection anatomy and slow, low-pressure, low-volume technique remain central.

Domain: Advanced Distinction **Difficulty:** Advanced **Type:** Advanced

Source: Goodman GJ, et al. Aspiration Before Tissue Filler - An Exercise in Futility and Unsafe Practice. Aesthetic Surgery Journal. 2022;42(1):89-101.

47. In a randomized, double-blind, split-face periocular study comparing PN with non-cross-linked HA, which broad result was reported?

- A. Neither treatment changed hydration or elasticity.
- B. Both groups improved, but the PN side showed a higher improvement rate in elasticity and hydration over time.
- C. HA caused permanent muscle paralysis.
- D. PN produced immediate structural volume equivalent to a cross-linked filler.

Correct answer: B

Rationale: The trial reported declining improvement over time in both groups, with the PN group showing higher improvement rates in elasticity and hydration.

Domain: Advanced Distinction **Difficulty:** Advanced **Type:** Advanced

Source: Lee YJ, et al. Comparison of the effects of polynucleotide and hyaluronic acid fillers on periocular rejuvenation: a randomized, double-blind, split-face trial. Journal of Dermatological Treatment. 2022;33(1).

48. Which conclusion is most appropriate from a 2025 randomized controlled trial of injectable recombinant humanized type III collagen for facial rejuvenation?

- A. The preliminary trial reported improved aesthetic scores, but results apply to the studied formulation and do not prove that every collagen-containing product or route is equivalent.
- B. Any topical product containing the word collagen can therefore be injected.
- C. Recombinant collagen permanently replaces all native collagen after one treatment.
- D. The trial proves that collagen boosters are substitutes for surgery in advanced laxity.

Correct answer: A

Rationale: The study reported improvement with the investigated rhCol III intervention, but clinical evidence is formulation- and protocol-specific and should not be generalized to every collagen product or route.

Domain: Advanced Distinction **Difficulty:** Advanced **Type:** Advanced

Source: Huang Z, et al. Translational Application of Recombinant Humanized Type III Collagen in Facial Rejuvenation: A Randomized Controlled Trial. *Journal of Cosmetic Dermatology*. 2025;24(11):e70529.

49. Which formulation matches the published 2024 clinical study of Karisma Rh Collagen FACE?

- A. Non-cross-linked high-molecular-weight HA, a recombinant collagen-1 alpha-chain polypeptide and carboxymethyl cellulose.
- B. Cross-linked HA, salmon PDRN and botulinum toxin type A.
- C. Calcium hydroxylapatite, PLLA and lidocaine only.
- D. Adipose-derived extracellular vesicles and tranexamic acid.

Correct answer: A

Rationale: The published formulation contained non-cross-linked HMW-HA, a human recombinant collagen-1 alpha-chain polypeptide and CMC.

Domain: Advanced Distinction **Difficulty:** Advanced **Type:** Advanced

Source: Di Rosa L, et al. New Regenerative and Anti-Aging Medicine Approach Based on Single-Stranded Alpha-1 Collagen for Neo-Collagenesis Induction. *Biomedicines*. 2024;12(4):916.

50. Why is an exosome label such as "5 billion particles" insufficient by itself to establish product quality or potency?

- A. Because extracellular-vesicle assessment also requires information about source material, separation and purity, size and molecular characterization, handling and storage, and functional evidence.
- B. Because particle count is legally prohibited in all countries.
- C. Because every particle measured is automatically a live stem cell.
- D. Because lower particle counts always produce better outcomes.

Correct answer: A

Rationale: MISEV guidance emphasizes multidimensional characterization and reporting. Particle concentration alone does not establish identity, purity, biological activity, stability or clinical efficacy.

Domain: Advanced Distinction **Difficulty:** Advanced **Type:** Advanced

Source: Welsh JA, et al. Minimal Information for Studies of Extracellular Vesicles (MISEV2023). *Journal of Extracellular Vesicles*. 2024;13:e12404.

Question blueprint and domain map

Domain	Questions	Count
Scientific foundation & mechanism	11, 12, 20, 24, 25, 29, 30	7
Indications & patient selection	1, 2, 13, 28, 33	5
Contraindications & precautions	4, 5, 6, 23, 37, 39	6
Treatment protocol & technique	7, 14, 15, 16, 21, 22, 26, 27, 31, 34, 35, 38	12
Complications & emergency management	8, 10, 17, 18, 36, 40	6
Post-care, consent & documentation	3, 9, 19, 32	4

Application emphasis: 30 of 40 MCQs are explicitly case- or application-based. Additional questions require interpretation rather than simple recall.

External references used only for Advanced Distinction questions

1. Goodman GJ, et al. Aspiration Before Tissue Filler - An Exercise in Futility and Unsafe Practice. *Aesthetic Surgery Journal*. 2022;42(1):89-101.
2. Lee YJ, et al. Comparison of the effects of polynucleotide and hyaluronic acid fillers on periocular rejuvenation: a randomized, double-blind, split-face trial. *Journal of Dermatological Treatment*. 2022;33(1).
3. Huang Z, et al. Translational Application of Recombinant Humanized Type III Collagen in Facial Rejuvenation: A Randomized Controlled Trial. *Journal of Cosmetic Dermatology*. 2025;24(11):e70529.
4. Di Rosa L, et al. New Regenerative and Anti-Aging Medicine Approach Based on Single-Stranded Alpha-1 Collagen for Neo-Collagenesis Induction. *Biomedicines*. 2024;12(4):916.
5. Welsh JA, et al. Minimal Information for Studies of Extracellular Vesicles (MISEV2023). *Journal of Extracellular Vesicles*. 2024;13:e12404.

Venus K-Glow Boosters

Result, Domain Scorecard and Retest Record | Version B

Assessment code	VA-KGLOW-B-2026.08
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Candidate and attempt details

Doctor full name		Employee ID	
Branch / city		Assessment date	
Assessment version	B	Examiner	
Attempt number		Retest version if applicable	

Score calculation

Section	Maximum	Score obtained
Core and applied MCQs	80	
Written clinical questions	20	
Core assessment total	100	
Advanced Distinction	5	

Final core percentage: _____ % Advanced score: _____ / 5

MCQ domain performance

Domain	Question numbers	Correct	Total	%
Scientific foundation & mechanism	11, 12, 20, 24, 25, 29, 30		7	
Indications & patient selection	1, 2, 13, 28, 33		5	
Contraindications & precautions	4, 5, 6, 23, 37, 39		6	
Treatment protocol & technique	7, 14, 15, 16, 21, 22, 26, 27, 31, 34, 35, 38		12	

Complications & emergency management	8, 10, 17, 18, 36, 40		6	
Post-care, consent & documentation	3, 9, 19, 32		4	

Final result category

Select	Result	Rule
[]	Pass with Distinction	Core score 90% or above and Advanced score at least 4/5.
[]	Pass	Core score 80% or above but distinction criteria not met.
[]	Retest Required	Core score 70-79%; targeted review and alternate-version retest.
[]	Retraining Required + Retest	Core score below 70%; retraining before complete retest.
[]	Invalid Attempt	Incomplete paper, misconduct or examiner-cancelled attempt.

Strong areas:

Areas requiring review:

Examiner comments:

Candidate signature	_____	Date	_____
Examiner signature	_____	Date	_____
Training Lead / Medical Director	_____	Date	_____

Remediation and retest record

Complete this page only when the result is Retest Required, Retraining Required + Retest, or an Invalid Attempt requiring a supervised repeat.

Original score		Original result	
Original version	B	Original date	
Required retest version		Retest date	

Areas reviewed and remediation completed

Competency area	Issue identified	Review / action completed
Scientific foundation & mechanism		
Indications & patient selection		
Contraindications & precautions		
Treatment protocol & technique		
Complications & emergency management		
Post-care, consent & documentation		

☐ Relevant slides reviewed ☐ Complete presentation repeated ☐ Manual reviewed

☐ One-to-one trainer discussion ☐ Case discussion ☐ Practical demonstration repeated

Trainer comments:

Retest outcome

Retest MCQ /80		Written /20		Core /100	
Advanced /5		Retest version		Final result	

Final decision / focused improvement plan:

Candidate signature		Date	
Examiner signature		Date	
Training Lead / Medical Director		Date	